TZEDEK DC
Legal Help for People in Debt

Board of Directors
Irvin Nathan, *Member*
Courtney Weiner, *Vice Chair*
A. Robert Bloom, *Treasurer*
Rahsaan Bernard, *Member*
Rachel Kronowitz, *Chair*
Willard Tom, *Member*
Peter Edelman, *Honorary Director*
Paul Berger, *Honorary Director*
Ariel Levinson-Waldman, *Founding President & Director-Counsel*
Ronald Glancz, z"l, *Permanent Member*

Advisory Council
Anthony Alexis
Roy Austin, Jr.
Nicole Austin-Hillery
Claude Bailey
Allyson Baker
Michael Baratz
Vivian Bass
Mathew Bruckner
Barry Coburn
Stephen Danzansky
Daniel Edelman
Andrew Eskin
Vicki Fishman
Marc Fleischaker
John Freedman
Shana Glickfield
Sally Greenberg
Andrew Herman
Mark Kahan
June Kress
Karen Ann Lash
Richard Lawch
Joshua Levy
Nancy Liebermann
Susan Liss
Stephen Marcus
Lorelie Masters
Carol Mates
Christopher McCannell
Dan Neustadt
Michael Nilsson
Kathryn Oberly
Ellen Oberwetter
BB Otero
Thorn Pozen
Harvey Reiter
Thomas Riesenberg
Seth Rosenthal
Robert Rozen
James Sandman
Nina Simon
John Sturc
Kenneth Trombly
Rebecca Vallas
Roger Zuckerman

The Honorable Rohit Chopra
Director, Consumer Financial Protection Bureau
1700 G Street, NW
Washington, DC 20552

Via Electronic Submission: www.regulations.gov

September 11, 2023

Re: Request for Information Regarding Medical Payment Products (CFPB Docket No. CFPB-2023-0038)

Dear Director Chopra:

Tzedek DC submits this comment in support of the efforts of the U.S. Consumer Financial Protection Bureau (CFPB), the U.S. Health and Human Services Agency (HHS) and the U.S. Treasury (collectively, the agencies) in understanding the prevalence, nature, and impacts of medical payments products, including whether these products allow healthcare providers to operate outside of a broad range of patient and consumer protections, displace hospitals' provision of financial assistance, lead to inflated or inaccurate medical bills, or otherwise harm patients' well-being.

Tzedek DC is an independent public interest center headquartered at the University of the District of Columbia David A. Clarke School of Law. Tzedek DC's mission is to safeguard the legal rights and financial health of DC residents with low incomes dealing with the often-devastating consequences of abusive debt collection practices and other consumer related issues. We at Tzedek DC carry out that mission with the goal of addressing racial gaps in wealth and equality. The racial wealth gap is especially wide in the District of Columbia. White DC households are estimated to have, on average, net assets of 81 times more than Black DC households, and 22 times more than Latino DC households. Tzedek DC offers direct legal and financial counseling services at no cost to low- and moderate-income DC residents struggling with debt and other consumer-related legal issues, engages in bilingual community outreach, and advocates for systemic reform in DC and nationally.

To what extent medical credit cards and loans may hamper financial assistance and access to benefits, and any options for regulators to reduce such barriers

The provision of medical credit cards to finance medical care and treatment may have several collateral, and unintended consequences, to the patient. First, the patient may in fact qualify under either their insurance or through their provider's

financial assistance program (FAP) for free or reduced cost out-of-pocket care. If a patient puts the bill on a medical credit card, they lose out on any opportunity to negotiate the price of the care or receive financial assistance. Second, as that patient may have been covered by insurance, they could be paying *more* for both the actual service as well as for any accrued interest through a credit card payment arrangement than if they simply made a claim with their insurance. Further, many providers offer zero interest payment plans that patients miss out on when using medical credit cards, which often have high rates of deferred interest. Third, financing the medical costs through a medical credit card functionally converts that debt from "medical debt" to credit card debt and any additional protections for medical debt, such as medical debts under $500 no longer being reported by Credit Reporting Agencies (CRAs) to a credit report, are lost to the patient.

An October 2015 study of the tax filings of more than 1,800 not-for-profit hospitals, published in *The New England Journal of Medicine,* concluded that only 29% offer uninsured patients discounts that match those negotiated by commercial insurers or Medicare. Further, the study noted that only 42% notify their patients about eligibility for charity care prior to trying to collect payment for their bills. A significant portion (between 11% - 28%) of written-off "bad debt" held by healthcare providers likely should have been free care based on the patients' income, but patients were either not screened properly or were not made aware of charity care/financial assistance program policies prior to their being stuck with the bill. Federal regulations should set minimum income guidelines for financial assistance across all providers and require preemptive screening of patients for assistance. For example, nonprofit healthcare providers should be required to provide free care for all patients earning up to 250% of the Federal Poverty Level (FPL) and reduced-price care to all patients earning up to 650% of the FPL. Regulators should also require that providers inquire into a patient's insurance and screen patients for financial assistance before offering them any medical payment products. Currently, these payment products are offered directly at the point of care when patients are most vulnerable. Existing assistance avenues should be exhausted before medical payment products are offered; even then, there should be a waiting period before such products can be offered to patients so they do not feel pressured to use these products.

Regulators should also require FAP information to be provided in multiple languages and forms of communication to be inclusive (i.e. braille, TTY-based telecommunications relay service, etc) and ensure that all patients can understand their potential eligibility and benefit from these policies.

How to ensure that patients retain their rights to challenge inaccurate bills regardless of payment method

Tzedek DC, in conversations with advocates and legislators, has discussed ways to reform debt collection practices, including the desire to prohibit hospitals from seeking to collect medical debt under the Fair Debt Collection Practices Act (FDCPA). Additionally, we have advocated for and are doing additional advocacy on how to curb other debt collection practices, such as wage and bank account garnishments and liens.

We have also discussed the mechanics of imposing conditions on hospitals that receive Medicare funding with legislators' offices in Congress to ensure that they are complying with financial assistance program policies. Nearly every provider in the US takes Medicare (and perhaps to a lesser extent, Medicaid) patients, and CMS can negotiate on behalf of those patients in a way those patients will never be able to themselves. Such negotiations may even provide advantages through better rates to patients who are either uninsured or underinsured, limiting the possible extent of their own medical debt.

With respect to medical credit cards, either these products should not be offered in the way they are currently – with very high interest rates and functionally converting medical debt to credit card debt – or they should be very clearly delineated as representing medical debt. This would help ensure existing protections for medical debt are not lost when medical payment products are used. If such products are to continue, regulations should

be implemented that clearly explain that payment of a bill on a medical credit card is not an acknowledgement of the amount owed and the patient does not waive their right to dispute the bill. If the patient does dispute the bill, the patient should be able to receive a full refund, including any interest paid on an inaccurate medical bill.

Further, the $500 voluntary reporting change the three major CRAs implemented regarding medical debt is a good start, but medical debt as a class of debt should not be reported on credit reports at all, as credit reports are meant to indicate a "reliable borrower" but owing for a medical emergency or chronic illness is a poor gauge of reliability. Colorado has already banned complete reporting of medical debt on credit reports; there is no reason this cannot be extended nationally.

Market-level inquiries and Agency-Specific Inquiries
1. What are the benefits, costs, and risks of medical payment products for consumers, healthcare providers, and companies offering these products?

For patients, the costs and risks likely outweigh the benefits. Many patients who are offered such payment products are already covered by insurance and may be eligible for coverage through their insurer; in practice, offering medical payment products seems to circumvent submitting claims to insurance, which for the provider can be more time-intensive and may result in a lower payment. Yet patients may be paying more for a service that should have been covered through their insurer, *and* are now responsible for debt that can be included on their credit report as a credit card debt owed (i.e. *not* medical debt) at what is typically a significantly higher interest rate than on a traditional credit card, 25%+ versus 15%+, respectively. Additionally, in studies on the use of such cards, it has been found to decrease access to credit, increase financial stress about medical bills, increase the likelihood of bankruptcy, and involve lengthy collection litigation.

Further, as mentioned above, patients may lose out on access to financial assistance when using medical payment products. Once a bill has been paid using such a payment product, the patient can no longer apply for financial assistance through the provider. Medical payment products usually have worse terms for consumers than any payment plans offered directly by the provider, which again the consumer misses out on by using such a product.

Consumers using these products have the ability to pay their bill immediately, but often feel pressured into using the payment products to do so even if they are not a good option for the consumer. Using these products eliminates all the medical debt credit reporting protections currently in place for consumers and increases the risk that a consumer will be sued over this debt and have a negative account for it on a credit report.

5. Are there specific populations (e.g., race, socioeconomic status, gender identity, sexual orientation, age, language, etc.) or geographic regions that experience disproportionately higher utilization of medical payment products?

Generally, in the District of Columbia, approximately 45% of residents are covered by Medicaid (i.e. approximately 300,000 residents), and according to the Kaiser Family Foundation (KFF), 95% of non-elderly Medicaid enrollees in DC are people of color.[1] The DC area is also home to a higher percentage of foreign nationals than the U.S. as a whole, with 15% versus 4%, respectively, according to the Urban Institute.[2] Given DC's Medicaid coverage rate of nearly half the jurisdiction's population, as well as the higher-than-average foreign-born population, it is **possible** that large portions of DC's population are vulnerable to medical payment products either due to language or cultural barriers (particularly coming from countries that have single-payer healthcare) or due to socio-economic status and a provider seeking to fast-track a payment process around an established insurance claim.

Specifically in our work, our clients have incomes below 400% FPL, and therefore in our experience, those who have used medical credit cards also have incomes below 400% FPL. Additionally, all of our clients who, to our knowledge, used medical payment products were women.

6. What are the health equity impacts of medical payment products and related billing and collection policies and practices?

For patients who opt to use medical payment products, they could face high, deferred interest on a medical credit card, which may also negatively impact their credit report and score. These consequences may make it more difficult for them to either pay for previously received care and/or be able to schedule and receive additional follow-up care. Some healthcare providers have explicit policies of turning patients away who have outstanding medical debt, which leads to worse health outcomes for patients. Even if patients can access care, the high rates of deferred interest on medical credit cards remains a significant source of anxiety for many patients, who were unaware of these terms and are unable to afford their bills. These products can also erode trust in our healthcare system, since patients often feel pressure to use them from their provider and then are hit with unfair and surprising terms. With respect to health equity, which is where everyone has a fair and just opportunity to attain their highest level of health, being foreclosed from seeking care due to previous medical debts is the antithesis of having a fair and just opportunity to attain the highest level of health.
In conversations with patients and advocates, we have heard from everyday Americans who have been faced with terrible medical emergencies only made worse by their dealings with the healthcare and insurance system afterwards. One such example includes a patient who experienced traumatic brain injury in a car accident resulting in a $1 million hospital bill. The protections of the Affordable Care Act unfortunately do not go far enough, as this patient's follow-up care costs and needs were hardly covered, forcing a filing for bankruptcy. To complicate matters further, due to the patient's ongoing financial difficulties, ongoing and necessary follow-up medical care was stopped by the provider.

Further, many Americans do not know how to read a medical bill and do not know how to dispute any inaccuracies. These inherently confusing practices can lead to patients ignoring bills that they cannot afford or are inaccurate. A major impact of these bills is on mental health – Americans overall are more worried about going into medical debt than of getting a serious illness. Once medical bills are sent to collections, it significantly negatively impacts patients' credit scores, even though it is an unreliable contributor to credit-worthiness. Worse, when patients are sued over medical debt, most cases end in default judgments for the provider. The current billing and collection landscape had serious negative impacts on patients' mental health, physical health, and credit.

ii. Do certain products or policies present opportunities to better serve members of underserved communities?

Clear, consistent, well-publicized and explained financial assistance policies would be of great benefit to tens of thousands of DC residents. Currently in the District there are eight Federally Qualified Health Centers (FQHCs) which provide services on a sliding scale, including at no cost to patients who qualify. According to the KFF, in 2021, DC's eight FQHCs provided nearly 900,000 patient visits (562,930 in-person clinical, and 332,563 virtual visits) across 84 service locations.[3] The current population of DC is approximately 670,000 residents.
There are seven major hospitals in DC, and according to the American Hospital Directory, reporting in May 2023, DC's hospitals reported 81,935 discharges (representing 570,669 "patient days") for 2022.[4] Clearly not all of these patients would qualify for financial assistance; however, it should be possible with dedicated materials, training, and sufficient staffing in hospitals' billing departments to ensure that patients who do or may qualify for financial assistance have the opportunity to speak with a professional objectively about their rights and benefits and what may be available to them, **prior** to any billing of services to the patient. DC's FQHCs saw

nearly 1 million patients virtually and in-person during the pandemic in 2021 and continue to screen patients and provide low-cost and free care, when appropriate; there is no reason DC's hospitals cannot provide the same level of care and screening for their patients as community health centers provide.
Additionally, we recommend reforming and/or strengthening existing policies to better serve members of underserved communities, including:

- Requiring FAPs to provide free care to patients under 250% FPL and discounted care to patients below 650% FPL, and to cover both insured and uninsured patients
- Requiring healthcare providers to preemptively screen patients for FAP eligibility
- Requiring hospitals presumptively to provide financial assistance to patients who receive means-tested benefits, including SNAP, WIC, Low-Income Home Energy Assistance Program (LIHEAP), and free or reduced-price school meals
- Prohibiting lawsuits to collect medical debt if the consumer is eligible for free or reduced-price care
- Capping the interest rate that can be charged on medical debts to 3% per year
- Prohibiting consumer reporting agencies from including medical debt in credit reports and require debt collectors to notify residents that medical debt will no longer be included in credit reports
- Capping hospital commercial prices at a certain percentage of the amount of Medicare prices

These policies would all work to cap rates of medical debt and ensure more patients are able to access much needed financial assistance.

CFPB

1. What actions should the CFPB consider taking to address problematic practices related to medical credit cards or loans, including debt collection and credit reporting practices?

Tzedek DC recommends the CFPB seek to limit the interest rate that is charged on medical credit cards, as these rates are typically several points higher than the top-end rates on regular credit cards (i.e. 25%+ v. 15%+). The rate should be capped at the same rate for all other credit cards. Medical credit cards are also typically offered through a deferred interest promotion, which many patients may not fully understand, resulting in a large, lump sum payment that may be particularly expensive and beyond the patient's ability to pay. Studies have shown that patients have paid more than $1 billion in interest alone on deferred interest on medical credit cards, signaling both the financial windfall of these products to industry as well as their negative financial consequences to the borrower/patient.

Tzedek DC further recommends that the CFPB issue guidance prohibiting the marketing of medical credit card to patients via their healthcare providers due to the costly nature of the products as well as the possibility that providers are circumventing the provision of free or reduced cost care to eligible patients. In lieu of a strict prohibition, we would recommend guidance that healthcare providers desiring to offer such payment products must receive training to ensure they are providing comprehensive information about legally mandated FAP and/or zero-interest repayment options *before* offering such products to patients, and that healthcare provider staff are well-versed and can answer patient questions related to costs, repayments, interest and deferred interest, and how the product and payment may impact their financial standing and credit report.

6. How can the CFPB use its authorities to ensure people with medical bills in collections, including medical payment product debt, are screened for eligibility for financial assistance and other benefits?

Healthcare providers should offer proper financial screenings of their patients to ensure their potential eligibility for any FAP, or charity care/free or reduced-price care. Receipt of established government benefits should be

considered, which in the District of Columbia includes SNAP, Medicaid, TANF, LIHEAP, housing assistance and shelter services, Interim Disability Insurance (IDA), Supplemental Security Income (SSI), Social Security Disability Insurance (SSDI), and WIC, amongst other DC-specific programs. An analysis of a patient's ability-to-pay up-front should clarify how the provider and patient should move forward with the treatment, although in no way should the determination result in no provision of care.

The CFPB can require CRAs to code medical payment products as medical debt on credit reports. The CFPB can also require that anyone promoting the use of medical payment products first discuss financial assistance and require screening for financial assistance before an application for a medical payment product can be approved.

8. When hospitals write off a patient's debt as uncollectible or "bad debt" and cease attempts to collect, do they notify patients that collection attempts will cease? Would patients benefit from such notifications, and would such notifications reduce hospital revenue?

Hospitals do not notify patients that collection attempts will cease when they write off a debt as uncollectible. Notifying patients of such would be a positive thing for the mental (and even physical) health of the patient. Patients would benefit from knowing that the financial burden is no longer present, since it would eliminate one significant source of anxiety and may propel a patient to seek further needed healthcare. It would also benefit patients' financial health, as they can plan how and where to dedicate their financial resources differently knowing that their medical debt is no longer an issue.

Treasury

1. What policy actions should Treasury consider taking to address problematic practices related to medical credit cards or loans, including debt collection and credit reporting practices, to conform with the existing tax laws and regulations pertaining to tax exempt hospitals?

Treasury should use its authority to limit the ability of tax-exempt hospitals from promoting medical payment products and related debt collection. Treasury should promulgate regulations to specify when hospitals can promote such payment products, and what associated disclosures are necessary in order for a hospital to remain tax exempt. Treasury should also specify that the use of such products in no way limits or changes the requirements of hospitals to provide financial assistance and *cannot* be counted as a community benefit. In doing so, Treasury should also set more specific eligibility criteria for financial assistance policies to ensure uniformity across the country. If hospitals fail to comply with these regulations, Treasury should limit the value of tax exclusions to the amount of financial assistance a hospital in fact provides.

2. Should a tax-exempt hospital's signing patients up for medical payment products be considered similar to a tax-exempt hospital's selling medical debt, such that the special rules that only exclude debt sales from being extraordinary collection actions if certain requirements are met would be applied to these payment products?

Yes, a hospital's signing patients up for medical payment products should be considered similar to a tax-exempt hospital's selling medical debt. The special rules should apply to more specific requirements needing to be met in order for a hospital to do so. This would help ensure more patients are screened for FAP eligibility before being offered a medical payment product and would help protect patients' rights as they relate to medical debt.

5. What are best practices for hospitals publishing and making patients aware of financial assistance programs (beyond compliance with the widely publicized requirements found in the section 501(r) regulations)?

In order to ensure that patients are aware of financial assistance programs, hospitals should preemptively screen every patient for eligibility. Patients should be notified verbally of the availability of financial assistance when they check-in for their appointments, and when they check-out. Hospitals should also clearly post financial assistance information on their website, on their general billing page. Every written communication regarding billing or collections that is sent to the patient should have information about financial assistance written in large, bolded text at the beginning and end of each communication. Further, this information should be widely available in all the languages most frequently spoken in the area.

8. Does the availability of medical payment products generally benefit the community or assist patients financially?

The potential for medical payment products to circumvent established health insurance coverage and result in the offer of payment for services which may be covered by insurance neither assists patients financially nor is a benefit to the community. Medical payment products such as medical credit cards offer deferred interest, the details of which may not be clear to patients at the time of the agreement, which can result in large expenses patients may not be able to afford. Additionally, the use of medical credit cards to pay for healthcare may result in damage to credit, as functionally the debt is viewed as “credit card” debt and not as medical debt.

Tzedek DC appreciates the opportunity to provide comments to the agencies on the matter of medical payment products.